Welfare

En analys från Swedish Subsidiarity

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Here is the English translation of your file:

Welfare: the solidarity paradox

The bureaucratic trap

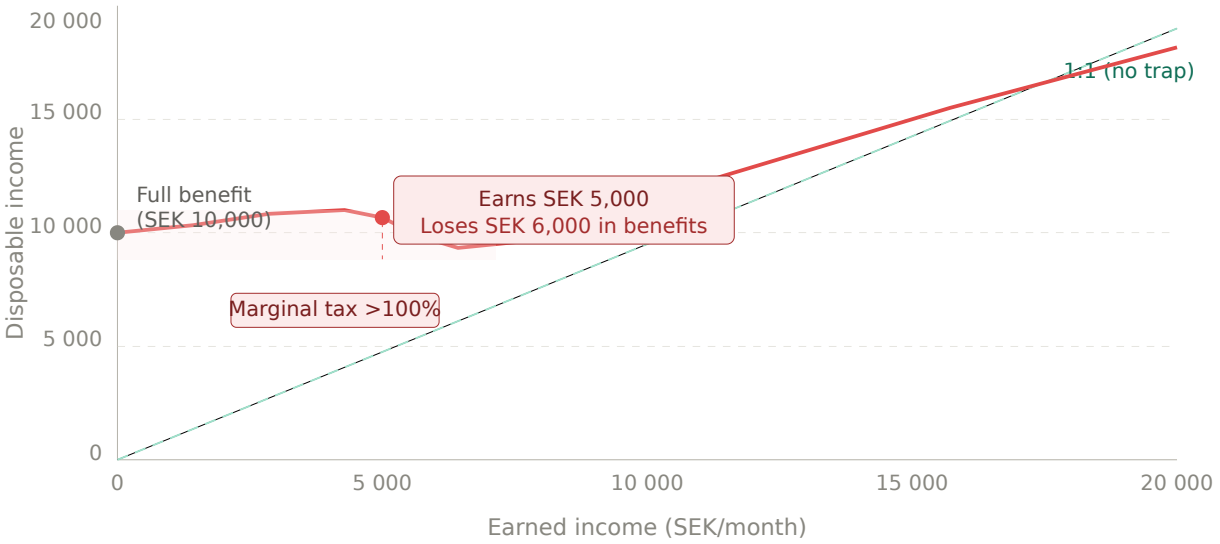
Sweden spends 30% of its GDP on social protection—about SEK 1,500 billion annually, the highest in the EU. Yet, 40% of applications for sickness benefits are rejected in the first instance, the average Swede spends 50 hours a year on welfare administration, and the system is now creating the insecurity it was designed to prevent.

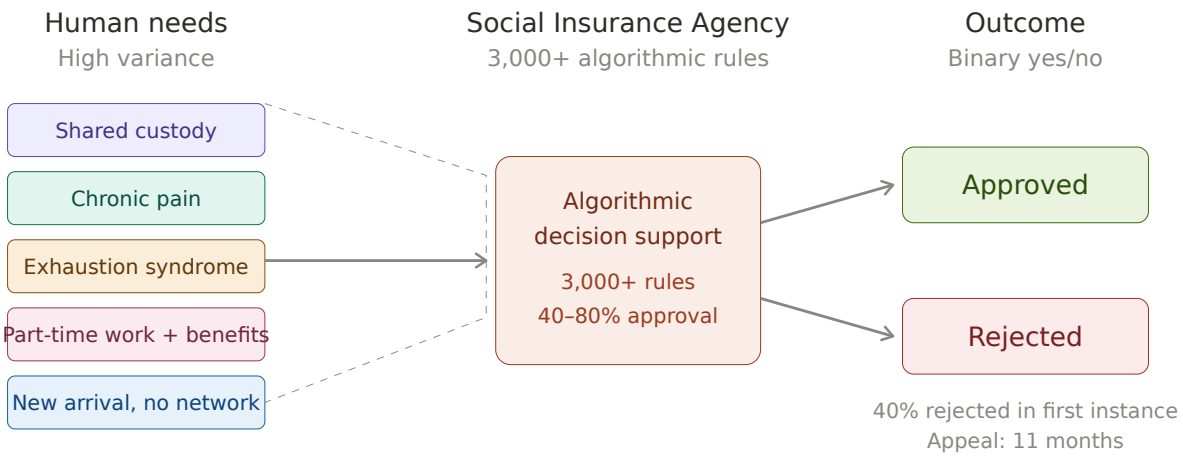
The paradox: 250,000 welfare professionals perform what families and neighborhoods once did naturally. The more the system professionalizes care, the more it eliminates caring. The more it standardizes support, the less it actually supports.

The cybernetic error: Human welfare has *high variance*—individual circumstances, family structures, cultural backgrounds, health statuses. The Swedish Social Insurance Agency (Försäkringskassan), with its 3,000+ algorithmic rules, is a control unit with *low variance*. Attempting to govern complex life needs through standardized algorithms violates Ashby's law. The result: escalating bureaucracy and weakened actual welfare.

Active system failures (in brief)

- **Algorithmic tyranny**: 3,000+ rules reduce complex lives to binary yes/no decisions. A single mother was denied parental leave benefits because the algorithm could not interpret shared custody—the appeal took 11 months.
- **Professional risk aversion**: Social workers spend 70% of their time on documentation, 30% on human contact. The system optimizes for legal protection, not for welfare.
- **Discretion without accountability**: The approval rate for sickness compensation varies from 40% to 80% among administrators at the same office—the outcome depends on chance, not on actual needs.
- **Medicalization of social problems**: "Exhaustion syndrome" (utmattningssyndrom) is now Sweden's leading cause of sick leave—a uniquely Swedish diagnosis that reflects systemic dysfunction, not individual pathology.
- **Poverty trap through means-testing**: A single parent who earns SEK 5,000 from part-time work loses SEK 6,000 in benefits. The system actively punishes the behavior it claims to encourage.





Ashby's law: low variety in the controller cannot match high variety in needs
Result: escalating bureaucracy and weakened actual welfare

Subsidiarity-based measures

Below are six action areas. Each area has its own page with concrete pilots, budgets, rule changes, and analysis of resistance.

Measure	Short description
→ Welfare rings	Groups of 10–20 households that detect needs early and offer light support (meals, childcare, companionship). Tax deduction of SEK 500/month per member.
→ Social prescribing	Doctors prescribe "20 hours of communal gardening" or "join a choir" instead of medication for loneliness, mild depression, stress.

Measure	Short description
→ Neighbor verification	For simple benefits (temporary housing support, crisis grants), two neighbors verifying the need is sufficient—processing in 48 hours instead of 6 weeks.
→ Personal welfare budgets	Long-term recipients receive a "welfare card" that combines several benefits into one monthly amount—the individual themselves prioritizes between rent, food, education.
→ Municipal welfare budgets	Municipalities receive per capita funding to design locally adapted support—national minimum level but local methods.
→ Social investment funds	Community foundations that finance preventive measures and measure returns in reduced future benefit dependency.

Implementation: from theory to practice

Phase 1 (1–3 years) – Pilots with welfare rings in 10 vulnerable areas. Train 1,000 doctors in social prescribing, 50,000 prescriptions the first year. Ten municipalities test neighbor verification for simple applications.

Phase 2 (3–7 years) – Budget delegation to municipalities with requirements for a minimum safety net. Personal welfare cards for long-term recipients. Social investment funds in three regions.

Phase 3 (7–15 years) – Transform "social assistance" (socialbidrag) into "community contribution" (samhällsbidrag) with an expectation of a return service (elderly companionship, homework help). Welfare banks in former bank branches as resource hubs.

Expected resistance and answers

Concern	Response
"Local variation creates inequality"	The current system already has massive geographical inequality in approval rates. Guidelines + the right to appeal protect against arbitrariness.
"Unions protect professional jobs"	Workers can become community facilitators—more qualified and meaningful than today's bureaucracy. Reduces burnout.
"My tax money shouldn't go to neighbors"	When people see that benefits support their own community (preschool teachers, elderly companionship), resistance decreases. Transparency is the key.
"Rules are rules – we can't trust neighbors"	Innovative pilots with regulatory exceptions show that community knowledge works better than bureaucracy for simple needs.

The Swedish advantage

Sweden has **high trust** (enables community-based methods), **digital infrastructure** (supports coordination), **tradition of popular movements** (cultural basis for mutual aid), **universalism** (reduces stigmatization), and **relative wealth** (enables investments in transition).

Conclusion: choose between escalating bureaucracy and real community

Today's welfare is not broken—it works exactly as it is designed. And the design is **architecturally flawed**: it optimizes for control, compliance, and legal security at the expense of human contact, contextual assessment, and actual well-being.

The choice is simple:

- Continue as today: More algorithms, more documentation, burned-out administrators, isolated benefit recipients, growing exclusion.
- Begin the transition: Welfare rings, social prescribing, neighbor verification, personal budgets, local investment funds.

Three things you can do tomorrow

If you are a municipal politician: Allocate SEK 500,000 for a pilot with welfare rings in a residential area with high insecurity. The model exists—start small.

If you are a doctor or nurse: Write a "social prescription" to the next patient with loneliness or mild stress—refer them to a gardening association, a choir, a walking group. See what happens.

If you are a citizen: Start a welfare ring with your neighbors. Just a group chat where you ask "who can babysit on Wednesday?" or "who has time to shop for grandma?". No grant needed—just initiative.

This analysis applies cybernetic principles (Ashby's law of requisite variety) to Swedish welfare. For deeper measures, click on the links above.

Welfare rings

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Summary: Groups of 10–20 households forming networks for mutual aid—early detection of needs (illness, unemployment, crisis), light support coordination (meals, childcare, companionship), and connection to professional services when truly needed. Members receive SEK 500/month in tax deductions for participating. The goal: to recreate the informal solidarity that professionalization has eroded, at a fraction of the cost of bureaucratic welfare.

The mechanism

Today, most welfare needs are discovered only when the individual crashes and turns to professional systems—the health center, social services, the Swedish Social Insurance Agency. By then, the crisis is already a fact. Neighbors, friends, and family often see the warning signs much earlier (isolation, weight loss, missed bills) but lack the structures to act.

The measure: A **welfare ring** is a loose grouping of 10–20 households in the same residential area or village that:

- Meets physically or digitally once a month (15–30 minutes)
- Shares information about who has the capacity to help ("I can babysit on Tuesdays") and who has a need ("I need a ride to the doctor")
- Offers light support without bureaucracy—meals, childcare, companionship, phone calls, help with simple household chores
- Acts as an early warning mechanism—if someone in the ring is not heard from for two weeks, another member calls to check in
- Connects to professional interventions when the need exceeds the ring's capacity (acute illness, domestic violence, severe mental illness)

The incentive: Each member receives a SEK 500/month tax deduction ("welfare deduction") for participating actively—attendance at meetings, reported contribution at least once a month. This recognizes that care work has value, even when not performed by professionals.

The cost comparison: A welfare ring with 15 households costs the state 15 × SEK 500 = SEK 7,500/month = SEK 90,000/year. For the same sum, you barely get half a social worker. The ring can deliver early

detection, companionship, childcare, meals, and social cohesion—things that professional systems are very expensive and often inefficient at.

Pilot design – 10,000 rings first

Pilot phase 1 (years 1-3): 10,000 welfare rings in 100 municipalities, focusing on:

- **Vulnerable areas** (according to the Police list) where both the need and the social capacity are high
- **Rural areas** where distances to professional systems are long
- **Senior housing** where loneliness is widespread
- **Suburban areas with a high proportion of newly arrived immigrants** where informal networks are often weak

Organizational model:

- The ring organizes itself—no application to an authority. Anyone can invite neighbors.
- To receive the tax deduction, the ring must register with the Swedish Tax Agency (Skatteverket) with a contact person and a list of members (personal identity numbers). No requirement for bylaws or forming an association.
- The contact person digitally certifies each month which members have been active (attended a meeting or performed a service). The Tax Agency pays the deduction automatically via the payslip.

State support:

- The ring receives a SEK 2,000 start-up grant for coffee, printing, a simple website, or renting a venue.
- The municipality provides a "ring coordinator" (1 per 100 rings) who can answer questions, share best practices, and connect to professional systems.

Evaluation after 3 years (compared to control areas without rings):

- Proportion who state they "have someone to ask for help" (social capital)
- Number of acute interventions from social services, ambulance, police (expected to decrease)
- Self-reported loneliness and insecurity
- Change in the municipality's costs for individual and family care

Budget and financing

Item	Annual cost (full rollout 10,000 rings)
Tax deduction (15 households × SEK 500 × 12 months × 10,000 rings)	SEK 900 million

Item	Annual cost (full rollout 10,000 rings)
Start-up grant (SEK 2,000 × 10,000 rings, first year)	SEK 20 million (one-time)
Ring coordinators (100 municipalities × 1 coordinator at SEK 500,000)	SEK 50 million
Digital platform (simple web app for registration, certification)	SEK 2 million
Evaluation (research)	SEK 5 million
Total first year	**SEK 977 million**
Annual operation after scaling up	**SEK 950 million** (excl. start-up grant)

Financing: Compared to the costs of social services (approximately SEK 100 billion/year), SEK 950 million is less than 1%. If the rings reduce acute interventions by just 5%, they have paid for themselves several times over.

Regulatory changes required

1. **New provision in the Income Tax Act (1999:1229)** – introduce a "welfare deduction" of SEK 500 per month for natural persons who actively participate in a registered welfare ring. The deduction is tax-free and does not affect other benefits.
2. **Ordinance on the registration of welfare rings** – the Swedish Tax Agency is tasked with establishing a simple digital register. Requirements: contact person, address, membership list. No bureaucratic control of "quality"—the rings are allowed to organize themselves.
3. **Exemption from the Secrecy Act** – In order for the rings to function as an early warning mechanism, members must be able to share certain information about each other (e.g., "Kalle hasn't been out of his house in a week"). Introduce an exception in the Public Access to Information and Secrecy Act (OSL 2009:400) that allows such sharing within the ring, as long as it is not spread further.

Expected resistance and answers

"This is legalizing insecurity – the state should not subsidize neighbors snooping."

Answer: The welfare ring is based on voluntariness and trust. No one is forced to participate. And "snooping" is the wrong word—it is about caring. In a society where everyone sits behind closed doors, loneliness is a massive problem. The welfare ring offers a structured path back to the natural care of the neighborhood.

"Fraud – people will take the SEK 500 without doing anything."

Answer: The risk exists, but the ring has an incentive to police itself. If a member never participates, why would the others want to keep them? The contact person only certifies active members. And SEK 500 is enough to motivate most people, but too little to make organized fraud profitable. Random checks can be carried out.

"The unions will protest – this threatens welfare jobs."

Answer: The welfare ring does not replace professional interventions—it complements them. Social workers should not cook dinner for lonely elderly people or babysit on a Tuesday evening. The ring does what professionals have neither the time nor the right competence for. On the contrary: the ring can free up professionals for what they are trained to do.

"I don't want to be dependent on my neighbors – I want rights."

Answer: The rights mentality is part of the problem. A society where everyone demands professional interventions for the slightest thing is a society that has lost the ability to care. The welfare ring does not offer a legal demand for help—it offers a community. The difference is crucial. Most prefer a kind neighbor over an administrator.

"This only works in homogeneous areas with high trust."

Answer: Then it is a good thing that Sweden has high trust generally. But the pilots are specifically designed to be tested in different areas—including socioeconomically vulnerable and ethnically mixed ones. Experiences from e.g., Botkyrka and Rosengård show that neighborhood networks can work even there, if they are given simple organization and modest support. If it doesn't work—then we have learned something.

Swedish examples to build upon

- **Neighborhood Watch (Grannsamverkan)** (the police)—a simple model where neighbors keep an eye on each other's homes. The welfare ring extends this to care.
- **Voluntary Resource Groups** (The Red Cross)—exist in many municipalities, but are often centrally controlled. The welfare ring is entirely self-organized.
- **The community center movement (Bygdegårdsrörelsen)**—a historical role model for local cooperation. Less active today, but the concept lives on.
- **"Support groups for relatives"**—exist within elderly care, but often require professional leadership. The welfare ring is more horizontal.

No Swedish model does exactly this—but fragments exist. The welfare ring gathers them into a simple, scalable idea.

What you can do tomorrow

If you are a municipal politician: Submit a budget request for SEK 100,000 to start a pilot with 10 welfare rings in a vulnerable area. Use the text above as a basis.

If you are a property owner or housing company: Put up a notice board in the stairwell: "Start a welfare ring – register your interest here." Offer a room once a month for free.

If you are a citizen: Invite your neighbors for a cup of coffee. Say: "I thought we could help each other out a bit—babysit, shop, call if someone is sick. No demands, just if we want to." You don't need to wait for politicians.

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Here is the English translation of your file:

Social prescribing

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Summary: Doctors prescribe community activity instead of medication—"20 hours of communal gardening," "join a choir," "walking group twice a week". Social prescriptions match patients with local resources via a digital platform. The goal: to address the root causes of loneliness, mild depression, and stress—conditions that are currently medicated even though they are fundamentally social. Half a billion SEK annually could be reallocated from pharmaceuticals to social interventions, with better health outcomes at a lower cost.

The mechanism

Today, millions of prescriptions are written for antidepressants, anti-anxiety medications, and sleeping pills for conditions often caused by loneliness, meaninglessness, physical inactivity, or social isolation. The drugs relieve symptoms but do not address the root causes. At the same time, there is an abundance of cost-effective, non-medical interventions—allotment gardens, choirs, walking groups, cooking meetups, language cafes—that no patient gets access to through the healthcare system.

The measure: Introduce **social prescribing** as a legitimate form of treatment in primary care.

- Doctors (or nurses, occupational therapists) assess that the patient's problems are largely due to social factors: loneliness, isolation, inactivity, meaninglessness.
- Instead of (or as a complement to) medication, the doctor writes a **social prescription**—a referral to a specific community activity for a set period (e.g., 8 weeks, 2 times/week).
- The patient receives a "social prescription card" that grants free entry or heavily subsidized participation (the municipality or region covers the cost).
- A **prescription reception** (physical or digital) connects the patient with local activities and follows up after the completed period.
- The activities are run by non-profit associations, educational associations, housing companies, or private actors—but are quality-assured according to a simple template (safe environment, trained leader, social interaction).

What can be prescribed (examples):

Activity	Expected effect	Cost per patient (8 weeks)
Allotment gardens	Physical activity, social contact, meaningfulness	SEK 400
Choir singing (community choir)	Social belonging, breath control, joy	SEK 600
Walking group 2 times/week	Physical activity, nature contact, conversation	SEK 0 (volunteer leader)
Cooking cafe (cooking together)	Social contact, nutrition, practical skill	SEK 300
Language cafe (for isolated newly arrived)	Social contact, language practice, belonging	SEK 200
Book club (library's)	Cognitive stimulation, social belonging	SEK 0
Writing circle (popular education)	Meaningfulness, self-expression, community	SEK 500

Follow-up: After 8 weeks, the doctor (or prescription reception) evaluates the effect using surveys (e.g., UCLA Loneliness Scale, WHO-5 Well-Being Index). If no effect—consider traditional treatment. If effective—renew the prescription or transition to independent participation without a subsidy.

Pilot design – 100 primary care centers first

Pilot phase 1 (years 1-3): 100 primary care centers in different parts of the country (large cities, smaller cities, rural areas) introduce social prescribing as a tested method.

Selection criteria for primary care centers:

- At least 2,000 listed patients
- Doctors who are positive toward non-medical interventions (identified via interview)
- Access to local non-profit associations or educational associations

Organizational model:

- Each primary care center gets a **prescription coordinator** (0.5 FTE, often a nurse or medical social worker) who maps local activities, quality-assures them, and connects patients.
- The region or municipality finances the subsidies (free tickets) for the patients.
- A digital platform (`socialarecept.se`) lists all quality-assured activities by postal code. The doctor clicks, prints the prescription, and the patient scans a QR code on site.
- Follow-up data is collected anonymously for research and quality improvement.

Goals for the first year:

- 50,000 social prescriptions
- 1,000 quality-assured activities
- 500 trained doctors/prescription writers
- Reduced prescription of antidepressants and anti-anxiety medications by 5% in the pilot regions

Evaluation:

- Change in self-rated loneliness, quality of life, physical activity
- Change in pharmaceutical dispensing (compared to control primary care centers)
- Healthcare contacts (emergency visits, admissions)—expected to decrease
- Cost comparison: social prescriptions vs. medications + healthcare visits

Budget and financing

Item	Annual cost (full rollout 100 primary care centers)
Prescription coordinators (0.5 FTE × 100 × SEK 500,000)	SEK 25 million
Subsidies (50,000 prescriptions × average SEK 300)	SEK 15 million
Digital platform (development + operation)	SEK 2 million
Training of doctors (500 × SEK 5,000 one-time)	SEK 2.5 million (one-time)
Evaluation (research)	SEK 3 million
Total first year	**SEK 47.5 million**
Annual operation after scale-up	**SEK 42 million** (excl. one-time costs)

Financing: The regions' pharmaceutical budgets are approximately SEK 25 billion annually. A reallocation of 0.2% is enough to finance the entire pilot. Socio-economically: if social prescriptions reduce sick leave, healthcare visits, and drug use by just a few percent, the reform is self-financing.

Regulatory changes required

1. **Amendment to the National Board of Health and Welfare's regulations (HSLF-FS)** – add social prescribing as a legitimate intervention in primary care. Today there is no prohibition, but no clear guidance. The regulation provides the right to use and invoice for social prescriptions.

2. **New code in the healthcare database** – to be able to follow up and reimburse the intervention, a unique intervention code (KVÅ code) for "social prescribing" is needed.
3. **Exemption in the Act on Pharmaceutical Benefits (2002:160)** – to clarify that social prescriptions are not a pharmaceutical benefit but a care measure, and thus are not affected by the high-cost threshold (the patient may pay a small out-of-pocket fee, but the subsidy is the main rule).

Expected resistance and answers

"Doctors shouldn't be dealing with gardening—it's not their job."

Answer: Doctors should do what best helps the patient. If loneliness and inactivity are the root cause of the problems, then a referral to an allotment garden is better medicine than a bottle of sertraline. The doctor does not need to garden themselves—just know the local resources and prescribe them.

"The pharmaceutical industry will lobby against it—they make billions from antidepressants."

Answer: Yes, probably. But public health should not be governed by commercial interests. Politicians and regions can make decisions based on evidence and cost-effectiveness, not on pressure. And several pharmaceutical companies already have social interventions in other countries—they can adapt.

"There is no evidence that social prescribing works."

Answer: Yes, the evidence is growing. The UK's "social prescribing" has been evaluated in several studies: reduced healthcare consumption (on average 28% fewer emergency visits), improved quality of life, cost savings of £1–2 per pound invested. Sweden is lagging behind, but we can learn from the NHS. The pilot creates Swedish evidence.

"Patients who need medication won't get less—this is an addition."

Answer: Yes, social prescriptions are a complement, not a replacement for acute or serious conditions. But for mild to moderate depression and anxiety, where the evidence for medication is weak, social prescriptions can be the first choice. This frees up medication for those who truly need it.

"Non-profit associations can't handle streams of referrals."

Answer: That is why the prescription coordinator is needed—a professional link between healthcare and civil society. The coordinator quality-assures, trains, and follows up, so that the associations only need to do what they are good at: welcoming people into community. The associations can also receive a small compensation per participant (e.g., SEK 50/visit) to cover extra work.

Swedish examples to build upon

- **"Green rehab"** (SLU, Alnarp)—nature-based rehabilitation for exhaustion syndrome. Works, but is small-scale and expensive. Social prescribing is simpler and cheaper.
- **Wellness grant (Friskvårdsbidrag)**—employers subsidize exercise. Why can't the healthcare system subsidize social community in the same way?
- **Norwegian "social prescriptions" (sosiale resepter)**—Norway is testing similar models, especially for young people with mental illness. Sweden can copy this.
- **NHS Social Prescribing (UK)**—2,500 "social prescribing link workers" in primary care, evaluated positively. Sweden has no equivalents—a gap to fill.

What you can do tomorrow

If you are a doctor or nurse: Write a note to a patient: "Join a walking group—meets Wednesdays at 14:00 by the library." Follow up in a month. You do not need a new law.

If you are a manager at a primary care center: Allocate SEK 100,000 to hire a prescription coordinator part-time for six months. Map local activities. Test with 50 patients.

If you are a municipal or regional politician: Propose introducing social prescribing as a pilot in primary care. Use the text above as a basis. Ask for an evaluation after two years.

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Neighbor verification

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Summary: For simple benefit applications (temporary housing support, funeral costs, crisis grants), it is sufficient for the applicant to provide two neighbors who verify the need. Processing time: 48 hours instead of 6 weeks. The risks of fraud are low because neighbors have strong incentives to be honest (social pressure, risk of exclusion from the community). The system is built on trust and local knowledge—and frees up administrators for complex cases where professional assessment is truly needed.

The mechanism

Today, almost all benefit applications (social assistance, housing allowance, income support, various crisis grants) require extensive processing: means testing, asset checks, bank statements, rent slips, sometimes home visits. For simple, low-risk situations—a single mother whose refrigerator has broken down and needs SEK 3,000 for a new one—this is grotesque overkill. Six weeks of processing for an urgent need.

The measure: Introduce **neighbor verification** as an alternative to traditional processing for grants under SEK 10,000 where:

- The need is obvious and time-critical (urgent food, broken household appliance, funeral)
- The applicant has no previous convictions for benefit fraud
- The applicant can provide two neighbors (or other people in the same residential area/village) who can confirm the need

The process:

1. The applicant fills out a simple digital application (2 minutes): name, personal identity number, what the money is needed for, two neighbors' names and phone numbers.
2. The system sends an automatic SMS link to the neighbors: "Do you know [Name]? Is it correct that they need [amount] for [purpose]? Answer YES/NO."
3. If both neighbors answer YES within 24 hours—the money is paid out directly via Swish within another 24 hours.
4. If any neighbor answers NO, or does not answer, the case proceeds to standard processing (6 weeks).
5. Spot checks: 5% of all neighbor-verified cases are reviewed retrospectively by an administrator. In case of suspected fraud—repayment demand and suspension from fast verification for 2 years.

Why neighbors are reliable:

- Social pressure: Lying to help someone cheat risks ostracism from the community.
- Reciprocity: Most people do not want to sabotage their neighbors, but neither do they want to participate in organized fraud.
- Low profits: The amounts are small (max SEK 10,000). Risking one's social reputation for a small sum is not rational for most.
- Neighbors have contextual knowledge that no administrator can match: They see if someone's washing machine is really broken or if it "happens" to break every month.

Pilot design – ten municipalities first

Pilot phase 1 (years 1-3): Ten municipalities with varying geography and population composition test neighbor verification for selected benefit types.

Which benefits are included in the pilot:

Benefit type	Max amount	Typical case
Urgent food account (social services)	SEK 2,000	Individual or family without food for the day
Repair of household appliance	SEK 5,000	Broken refrigerator, stove, washing machine
Funeral costs (remission)	SEK 10,000	Relative lacks funds
Temporary housing support	SEK 8,000	Rent debt threatening eviction (max once per 12 months)
Crisis grant (domestic violence, fire, flood)	SEK 10,000	Urgent move, compensation for destroyed clothes/furniture

Organizational model:

- The municipality's social services or income support office is responsible for implementing the system.
- A digital platform (simple web app or SMS-based) is connected to the municipality's existing case management.
- One administrator (0.2 FTE per municipality) is responsible for spot checks and handling deviations (NO answers, missing answers).
- Payment is made via Swish or Plusgiro.

Exclusion criteria (may not use neighbor verification):

- The applicant has previously been convicted of benefit fraud (within 5 years)
- The applicant has used neighbor verification more than 3 times during the past year (risk of repetition)
- The neighbor is a family member or cohabitant (conflict of interest)

Evaluation:

- Processing time (median, 90th percentile)
- Proportion of applications going through neighbor verification (target: 30% of all simple benefits)
- Proportion of NO answers (expected low – <5%)
- Proportion of fraud discovered in spot checks (expected very low – <1%)
- Applicant satisfaction (survey)
- Staff experience of workload (freed-up time for complex cases)

Budget and financing

Item	Annual cost (10 pilot municipalities)
Digital platform (development + operation)	SEK 500,000 (one-time) + SEK 100,000/year
Administrator resource (0.2 FTE × 10 × SEK 500,000)	SEK 1,000,000
Spot checks (5% of cases, 20 min per case)	SEK 100,000 (estimate)
Staff training (1 day × 10 municipalities)	SEK 50,000 (one-time)
Evaluation	SEK 200,000
Total first year	**SEK 1.85 million**
Annual operation after scale-up	**SEK 1.1 million** (excl. one-time costs)

Financing: Within existing social service budgets. Because neighbor verification frees up administrator time (6 weeks → 48 hours), the costs per case decrease dramatically. Even with spot checks, neighbor verification is likely cheaper than traditional processing.

Regulatory changes required

1. **Amendment to the Social Services Ordinance (2001:937)** – add a section: *"For grants of a maximum of SEK 10,000, the municipality may apply a simplified processing where the need is*

verified by two residents in the vicinity who are not family members. Decisions must be made within 48 hours."

2. **GDPR adaptation** – The neighbors' personal data (name, phone number) is processed to verify needs. This requires legal support (Article 6.1 e – public interest). This already exists to some extent but is clarified in the ordinance.
3. **Exemption from the Secrecy Act** – Normally, social services may not disclose information about individuals to neighbors. Here, the applicant themselves provides the neighbors—no information is disclosed by the authority. No secrecy problem.

Expected resistance and answers

"Neighbors can lie – this opens up for massive fraud."

Answer: The risk exists, but it is small. Neighbors who systematically lie risk exclusion from the community, social stigma, and ultimately a police report for fraud. The amounts are small (max SEK 10,000). The incentives for fraud are low. And spot checks catch most attempts. Experiences from other countries (e.g., microloans with neighbor verification) show very low rates of fraud.

"Neighbors might feel forced to say yes – social pressure."

Answer: The applicant must provide the neighbors themselves. If the relationship is such that the neighbor cannot say no, then it is an unsuitable neighbor to use. The system encourages the applicant to choose people who can be honest. The neighbor can always answer NO anonymously (the SMS only asks YES/NO, no name is visible to the applicant). And NO answers do not lead to any sanction against the applicant—only to standard processing.

"This is a class issue – poor people do not have neighbors with resources to verify."

Answer: Neighbor verification is not about financial resources but about proximity and trust. An unemployed neighbor can just as easily confirm that the refrigerator is broken as a rich one. On the contrary: in socio-economically weak areas, neighborhood cooperation is often stronger because people are more dependent on each other. The pilot is specifically meant to test this.

"The administrators lose control and status."

Answer: On the contrary—they are freed from trivial, low-risk cases and can focus on complex cases where their expertise is truly needed (addiction, honor-based violence, psychiatry). Many administrators experience today's quantitative targets as stressful. Neighbor verification reduces the workload.

"Are there any international examples?"

Answer: Yes. Microloan organizations like Grameen Bank use neighbor verification ("solidarity lending") with very low default rates. In social work, "community verification" exists in several countries, e.g., for disaster relief. Sweden simply hasn't tested it. The pilot can make Sweden a pioneer.

Swedish examples to build upon

- **Swish payments from authorities** – several municipalities already use Swish for quick payouts (e.g., urgent food bags). The technology exists.
- **Neighborhood Watch (Grannsamverkan)** (the police) – shows that trust-based neighbor monitoring works in Swedish residential areas.
- **The Swedish Social Insurance Agency's digital services** – simple application for certain benefits (e.g., dental care allowance). Neighbor verification is the next step.
- **Norwegian "naboverifisering"** – Norway is testing similar models in social services, especially for young adults. Sweden can emulate this.

What you can do tomorrow

If you are a social worker or unit manager: Propose an internal pilot: Choose a benefit type (e.g., urgent food account) and test with 10 cases where the applicant brings a neighbor who certifies the need. Measure the time. Evaluate after one month.

If you are a municipal politician: Submit a proposal to introduce neighbor verification for grants under SEK 10,000 in your municipality. Use the text above as a basis. Ask for a report after six months.

If you are a systems developer: Build a simple prototype: an SMS interface where the applicant enters the neighbors' numbers, the neighbors receive a request and answer YES/NO, and upon two YESes, a Swish payment is sent automatically. The source code can be open for all municipalities.

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Personal welfare budgets

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Summary: Long-term recipients of multiple benefits (income support, housing allowance, maintenance support, introduction benefit, etc.) receive a "welfare card" that combines all parts into a single monthly amount. The card is loaded with a fixed sum based on the household's size and needs (but is higher than the sum of the benefits because administration and control cost money). The recipient decides how the money is distributed among rent, food, electricity, childcare, clothes, education, and savings. Unused funds can be saved or used for approved investments (e.g., driver's license, vocational training). The goal: dignity, flexibility, and incentives to work and save, instead of today's poverty trap where every extra krona earned leads to withdrawn benefits.

The mechanism

Today, many long-term benefit recipients live in a **poverty trap** where the system actively discourages work and savings. A single parent receiving income support, housing allowance, maintenance support, and possibly introduction benefit faces marginal taxes of over 100%: every hundred kronor earned from part-time work leads to benefits being reduced by more than a hundred kronor. The result: no rational person works. The system locks people into exclusion.

The measure: Introduce **personal welfare budgets** (also called "welfare cards" or "combined benefits") for long-term recipients who meet the following criteria:

- Have been dependent on income support or other means-tested benefits for at least 12 months (which signals that they are stuck in the system)
- Lack addiction or other problems that require special case management (in such cases, traditional models are retained)
- Can manage their own budget (assessed by a social worker, but the presumption is that most adults can)

What the card contains:

The card is loaded every month with an amount based on:

- Household size (adults, children)
- Housing cost (rent or fee, up to a ceiling)

- Special needs (e.g., dietary supplements in case of illness, travel to labor market programs)

The amount is **higher** than the sum of the benefits the person would otherwise receive—because administration and control are dramatically reduced (no monthly means testing, no arbitrary deductions). Experiences from similar systems show that one can be 10–20% above the benefit level and still save money for society.

What the card can be used for (standard):

- Rent and electricity (can be paid automatically)
- Food and household items
- Childcare, leisure activities for children
- Clothes, shoes
- Public transport, fuel (reasonable level)
- Medicines, dental care (outside the high-cost threshold)
- Savings (up to SEK 1,000/month, must not exceed a 6-month buffer)

What the card can be used for (after approval):

- Education (vocational courses, driver's license, computer license)
- Investments in work tools (computer, work clothes)
- Moving costs (to take a job in another location)

What the card may NOT be used for (blocked):

- Alcohol, tobacco, gambling
- Luxury goods (designer clothes, expensive restaurant visits—but normal restaurants are fine)
- Travel abroad (exceptions: funerals, acute family needs)

Incentives for work:

- Every krona earned from work **does not affect** the card's amount during the first 6 months. Thereafter, the card is reduced by 50 öre per earned krona (marginal tax of 50% instead of today's 100%+).
- Unused funds on the card (what is not used during the month) can be saved and used for approved investments, or taken out as a cash bonus after 12 months (max SEK 5,000). This creates an incentive to be frugal—the opposite of today's "use-it-or-lose-it".

Pilot design – 5,000 people first

Pilot phase 1 (years 1-3): 5,000 long-term benefit recipients in 10 municipalities get access to a personal welfare budget. The selection is made by lottery among qualified individuals (to enable a randomized control group).

Selection criteria:

- Income support for at least 12 months (continuous or with short breaks)
- Age 25–60 years
- No ongoing addiction investigation or LVU placement (Care of Young Persons Act) (such cases retain traditional case management)
- Can read and write Swedish (or with the support of an interpreter)

Organizational model:

- Each pilot municipality appoints a "budget coach" (1 per 200 participants) who informs, helps with technical issues, and supports budget planning if necessary.
- The card is administered via an existing platform (e.g., bank card linked to a digital account, similar to a payroll card). Several banks already offer "social cards"—these can be procured.
- Payment is made monthly automatically, based on housing cost and household size (updated every 6 months).
- Spot checks: 2% of transactions are reviewed automatically (blocked categories). In case of suspected fraud—the card is frozen and the case reverts to traditional case management.

Evaluation (compared to a randomized control group):

- Proportion moving from benefits to work (primary efficacy endpoint)
- Time to work (months)
- Self-reported quality of life, health, perceived dignity
- Administrative costs per person (expected to decrease by 50–70%)
- Incidence of fraud and abuse (expected to be low)

Budget and financing

| Item | Annual cost (5,000 participants) | | |------|----------| | | Higher benefit amount (15% above normal level, average SEK 3,000/month × 5,000 × 12) | SEK 180 million | | Budget coaches (5,000/200 = 25 coaches at SEK 400,000) | SEK 10 million | | Card administration (bank fee, platform, SEK 200/person/year) | SEK 1 million | | Spot checks (administrator time) | SEK 0.5 million | | Evaluation (research) | SEK 2 million | | **Total first year** | **SEK 193.5 million** | | **Annual operation after scale-up** | **SEK 190 million** (excl. evaluation) |

Financing: Today's costs for income support, housing allowance, maintenance support, and introduction benefit for 5,000 people are approximately SEK 250–300 million (depending on composition). Even with a 15% increase, the pilot is below current costs—because administration and control decrease drastically. If only 10% of the participants go to work (and thus leave the benefit systems), the pilot is self-financing.

Regulatory changes required

1. **Amendment to the Social Services Act (2001:453)** – add a new section: *"The social welfare committee may, after individual assessment, grant income support in the form of a personal welfare budget paid monthly on a special card. The budget shall be higher than the combined benefit level by at least 10 percent, corresponding to administrative savings."*
2. **Amendment to the Housing Allowance Ordinance (2001:933)** – for people with welfare cards, the housing allowance becomes a component of the card (no separate application). This requires an exemption from the main rule.
3. **Amendment to the Income Tax Act** – to avoid the card's payouts being taxed as income. They remain tax-free benefits, but since they are not means-tested monthly, this must be clarified.

Expected resistance and answers

"This is a waste – people will buy booze and gambling with the money."

Answer: That is why certain categories are blocked on the card. But even if a few abuse the system, it is better that 95% gain dignity and flexibility than that 100% are suffocated by bureaucracy. Experiences from similar systems (e.g., Canada's "cash transfers" for the homeless) show that the vast majority use the money wisely—often more wisely than social workers do.

"Municipalities lose control – they need to be able to assess the need every month."

Answer: The needs of a long-term benefit-dependent person do not change dramatically from month to month. Annual reassessment is sufficient. The time and energy freed up when administrators are relieved of monthly forms can be spent on helping people into work instead of checking their receipts.

"This is a hidden cut – people get less than today."

Answer: On the contrary—we propose an increase of 10–20% compared to current benefit levels. But even if the sum is the same, the recipient gets the opportunity to prioritize for themselves. Today, a single mother is forced to spend exactly SEK 4,700 on food (social services' standard amount) even if she would rather spend SEK 4,000 on food and SEK 700 on a training course for her child. The flexibility is worth at least as much as the money.

"People who cannot manage their finances will end up on the street."

Answer: That is why the pilot is voluntary and aimed at people who are deemed able to manage a budget. Those who obviously cannot (addiction, severe mental illness, dementia) retain traditional models with a trustee. And budget coaches are available for those who need support—a resource that today's system lacks.

"Are there any international examples?"

Answer: Yes. Canada's "Ontario Works" tested personal budget cards with positive results (reduced administration, higher satisfaction). The UK's "Universal Credit" is in practice a personal budget, although the implementation has been problematic. Finland's basic income experiment showed that people do not abuse freedom. Sweden is lagging behind—the pilot can provide Swedish evidence.

Swedish examples to build upon

- **Income support with a "reasonable standard of living"** – standard amounts already exist today. The welfare card is an extension of the same logic, but with individual distribution.
- **Payroll cards** – many employers already use prepaid cards for staff. The same technology can be used for benefits.
- **Activity support (The Swedish Public Employment Service)** – paid monthly without requiring receipt accounting. It works. Why can't income support do the same?
- **Denmark's "kontanthjælp" (cash benefit) with a job plan** – Denmark has simpler models, but still detailed regulation. Sweden's pilot can be even simpler.

What you can do tomorrow

If you are a social worker or unit manager: Identify 10 long-term benefit recipients who you assess would be able to handle a personal budget. Ask them: "If you got to decide for yourself over a pot of money every month, would you be able to handle it?" Test informally with an agreement (the municipality pays a fixed amount, the person accounts for it afterwards). Evaluate after three months.

If you are a municipal politician: Submit a proposal for a pilot with personal welfare budgets for 100 people. Use the text above as a basis. Ask for a randomized control group to be able to evaluate the effect on work and quality of life.

If you are a systems developer or fintech entrepreneur: Build a prototype for a welfare card with category blocking, monthly reloading, and a simple user app. Offer municipalities to test it for free for six months.

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Municipal welfare budgets

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Summary: Municipalities receive a consolidated welfare budget per capita (based on socio-economic needs) instead of earmarked government grants and minutely regulated programs. National minimum levels guarantee that no one falls below the poverty line, but methods, priorities, and organization are determined locally. The goal: allow different municipalities to test different solutions—basic income, service vouchers, community grants, social investment funds—and create a Swedish evidence base for what actually works in different local contexts.

The mechanism

Today, Swedish welfare is governed by a complex mix of national laws, ordinances, regulations, and earmarked government grants. Municipalities are responsible for implementation but have very little freedom to adapt methods to local conditions. A municipality with high unemployment and a municipality with an aging population are forced to use the same program templates—even though the needs are fundamentally different.

The measure: Introduce **municipal welfare budgets**—a reform in three steps:

1. **Merge earmarked government grants** (income support, housing allowance, elderly care, disability support, labor market interventions, etc.) into a **one-dimensional needs-based transfer** per municipality.
2. **Remove detailed regulations**—national goals ("reduce child poverty," "increase the proportion in work") remain, but the methods to achieve the goals are determined by the municipal council.
3. **Introduce evaluation and accountability**—each municipality must annually report how the budget was used and what results were achieved. In case of repeated failures, the state can revoke the delegation (sanction).

What the municipality is allowed to do (examples of local variations):

Municipality type	Possible solution	Why locally adapted?
Large city with high unemployment (Malmö, Södertälje)	Basic income or job guarantee for young people	Traditional measures have failed, need for radical tests

Municipality type	Possible solution	Why locally adapted?
Sparsely populated municipality with aging population (Älvdalen, Dorotea)	Service vouchers for the elderly (choose your own home care, food box, neighbor help)	Long distances, few private alternatives—empower the elderly
Suburban municipality with high segregation (Botkyrka, Rinkeby-Kista)	Community grants to local associations that run employment activities	Professional systems do not reach people—associations do
University city (Uppsala, Lund)	Investment support for cooperative student housing and psychosocial support	Young adults fall through the cracks—flexible solutions are needed

National minimum levels (guarantees that no municipality may fall below):

- No person shall have a disposable income below the national standard for income support (unless other support is available)
- Elderly care shall guarantee a minimum level of service (exact level determined nationally)
- Children in economically vulnerable households shall have access to at least one organized leisure activity
- Emergency social services (domestic violence, homelessness, addiction) must be available every day

Evaluation and learning:

- A national knowledge center (lifted from the National Board of Health and Welfare) collects data from all municipalities, analyzes what works (and what does not), and spreads best practices.
- Each municipality must budget at least 0.5% of the welfare budget for independent evaluation.
- After 8 years, a statutory review is conducted: which local models should be scaled up nationally? Which should be abandoned?

Pilot design – 20 municipalities first

Pilot phase 1 (years 1-4): 20 municipalities (representative of different municipality types) receive a delegated welfare budget with exemptions from detailed regulations. The pilot municipalities sign an agreement with the government specifying national minimum levels and evaluation requirements.

Selection criteria:

- Geographical distribution (northern, central, southern Sweden)
- Varying population base (large city, medium-sized city, rural area, sparsely populated area)
- Varying socio-economic challenges (unemployment, aging, segregation, integration problems)

- Political will—the municipality must apply and show that it has the capacity to implement the reform

Budget model:

- Each pilot municipality receives a **per capita compensation** based on:
  - Number of inhabitants (base)
  - Socio-economic needs index (unemployment, education level, ill health, proportion of foreign-born)
  - Geographical disadvantages (sparseness, long distances)
- The compensation replaces all previous government grants for the welfare areas (income support, elderly care, disability, labor market, childcare? – boundaries to be discussed). The municipality retains its tax capacity unchanged.
- The compensation is adjusted annually for inflation and population changes.

Examples of annual compensation (the pilots):

Municipality	Type	Inhabitants	Needs index	Compensation (SEK million)	Compensation per capita (SEK)
Malmö	Large city, high unemployment	350,000	1.25	4,375	12,500
Dorotea	Sparsely populated, aging	1,300	1.80	23	17,700
Botkyrka	Suburb, segregation	95,000	1.35	1,282	13,500
Uppsala	University city	240,000	0.95	2,280	9,500

Evaluation (independent researchers):

- Compare pilot municipalities with matched control municipalities (which retain traditional governance)
- Outcome measures: unemployment, child poverty, quality of life for the elderly, homelessness, cost development per inhabitant
- Interviews with citizens, users, administrators, and politicians
- Annual reports are published and audited by the Swedish National Audit Office (Riksrevisionen)

Budget and financing

Item	Annual cost (20 pilot municipalities)
Compensation to the municipalities (based on current government grants + tax equalization, no change in total level)	SEK 0 (reallocation)
Evaluation (research, data collection)	SEK 15 million
National knowledge center (lifted from the National Board of Health and Welfare)	SEK 10 million
Municipalities' own evaluation costs (0.5% of budget)	approx. SEK 30 million (total)
Total first year	**SEK 55 million**

Financing: Within existing government grants for welfare (approximately SEK 200 billion annually). SEK 55 million is 0.03%—a negligible proportion to test a potentially transformative reform.

Regulatory changes required

1. **Amendment to the Social Services Act (2001:453)** – add a new section: *"The government may, upon application, grant a municipality an exemption from the detailed regulations in this act to test alternative forms of welfare provision. The exemption may cover a maximum of 20 municipalities and must be evaluated after four years."*
2. **Amendment to the Act on Government Grants to Municipalities** – replace earmarked grants with a needs-based flat rate for the pilot municipalities.
3. **Ordinance on national minimum levels** – the government establishes which guarantees all municipalities (including pilots) must meet, e.g., national standard for income support, minimum staffing in elderly care.

Expected resistance and answers

"Municipalities will lower services to save money."

Answer: National minimum levels prevent a race to the bottom. And since the compensation is needs-based, municipalities have no incentive to skimp—on the contrary, if they deliver poor results, it shows in the evaluation and risks the state revoking the delegation. Most municipalities want to do a good job, not minimize costs.

"Inequality between municipalities will increase."

Answer: The current system is already deeply unequal—the differences in the quality of elderly care between rich and poor municipalities are enormous. Our model compensates for socio-economic needs, which reduces inequality. And by allowing different solutions, poor municipalities can test innovative, cost-effective methods that are currently prohibited.

"The state loses control and cannot guarantee equivalence."

Answer: Equivalence is not the same as uniformity. Two municipalities can reach the same goals (low unemployment, satisfied elderly care) using completely different methods. The state's role is to set goals and evaluate results—not to dictate methods. This is called trust-based management, and it has been an official policy for several years without being implemented. Now we are making it a reality.

"Bureaucrats and trade unions will oppose it—they lose influence."

Answer: Some will do that. But many municipal officials are frustrated by today's regulatory hassle and would welcome greater room to maneuver. Trade unions can gain from their members getting more qualified, varied tasks instead of being form-fillers. The transition must be handled carefully, with training and clear career paths.

"Are there any international examples?"

Answer: Yes, several. The UK's "local authority flexibility" (some delegation, but incomplete). Finland's "social welfare pilot regions" (positive experiences). Canada's "Ontario Works integration" (mixed results, but learning). Sweden's strong municipal self-government is a unique foundation—we can go further than other countries.

Swedish examples to build upon

- **Municipal self-government (the constitution)** – Sweden already has a unique position with strong municipalities. This is underutilized today.
- **Delegation trials in elderly care** – several municipalities have tested "free years," "short-term housing," "user influence" with good results. But they have had to fight against national rules.
- **LOV (Act on System of Choice in the Public Sector)** – gives municipalities the right to introduce freedom of choice in care. The same logic could be extended to the entire welfare sector.
- **Norway's "free municipality trials" (frikommuneforsøk)** – Norway has had trials with deregulated municipalities since the 1980s. Experiences are overwhelmingly positive (more innovation, more satisfied users). Sweden is lagging behind—time to catch up.

What you can do tomorrow

If you are a municipal commissioner or municipal director: Send a letter to the government and SALAR (Swedish Association of Local Authorities and Regions) expressing interest in becoming a pilot municipality in a welfare budget reform. Gather commissioners from several parties—the broader the political support, the better.

If you are a Member of Parliament or state secretary: Present a bill for a legislative amendment that enables pilots with municipal welfare budgets. Use the text above as a basis. Push for Sweden to become the best in the Nordic region at municipal welfare experimentation.

If you are a researcher or evaluator: Contact SALAR and offer to design a randomized control group study for the pilots. Good evaluation is crucial for the reform to be scaled up.

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Social investment funds

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Summary: Community foundations that finance preventive social interventions and measure returns in reduced future benefit dependency, fewer healthcare interventions, and higher quality of life. The fund invests in projects that currently do not receive public funding because they are "preventive"—the costs are incurred now, but the savings come several years later. Through social investment funds, zero-sum thinking is broken: every million invested preventively can save several millions in future acute interventions. The goal: to create a system that rewards prevention instead of merely managing crises.

The mechanism

Today, public budgets are divided into silos (social services, healthcare, labor market, schools) with short-term annual budgets. This creates a systemic flaw: no one has an incentive to invest in preventive measures that yield returns over several years and across multiple silos. An intervention that reduces juvenile delinquency saves money for social services, the police, prosecutors, the prison and probation service, and healthcare—but none of these authorities get to share in the savings.

The measure: Introduce **social investment funds**—independent community foundations that:

- Receive start-up capital from the state, region, municipality, and/or private donors
- Invest in preventive projects that have a high potential to reduce future public expenditures
- Carefully measure the effects (with randomized control groups or meticulous comparisons)
- Reinvest the savings into new preventive interventions—a self-reinforcing cycle

What the fund can finance (examples):

Intervention	Target group	Expected savings	Time horizon
Family therapy (home visits, parental support)	Young children in at-risk families	Reduced foster care placement (saves SEK 1–2 million/year per child)	3–5 years

Intervention	Target group	Expected savings	Time horizon
Supported employment (individualized job support)	Long-term sick-listed individuals with mental illness	Reduced sickness benefits, increased tax revenue	1–2 years
Debt counseling + savings (individual development accounts)	Low-income earners	Reduced reliance on social assistance, increased self-sufficiency	2–4 years
Building more youth activities on evenings/weekends	Youth in vulnerable areas	Reduced juvenile delinquency, reduced costs for vandalism	1–3 years
Housing support + job coach (intensive package)	Newly arrived immigrants with low education	Reduced introduction benefits, faster entry to work	2–3 years

The investment model:

- The fund grants a loan (or a grant with repayment requirements) to the project. The loan must be repaid if the expected savings are realized (according to predetermined metrics). If the project fails, the loan is written off—the risk is shared between the fund and the provider.
- The savings are calculated by independent evaluators: comparing the costs for the target group with a control group (or historical data). The difference in public expenditures (social assistance, healthcare, prison and probation service, etc.) constitutes the "social return".
- A portion of the savings (for example, 50%) is repaid to the fund, while the rest remains in the public sector. In this way, both the fund and the municipality/region/state get a piece of the pie.

Example: A family therapy intervention costs SEK 200,000. Without the intervention, there is an 80% probability that the child would be placed in a foster home for two years, costing SEK 1.6 million. With the intervention, the child is not placed in care at all. The savings for society: SEK 1.6 million – SEK 200,000 = SEK 1.4 million. The fund gets back SEK 700,000 (50%), which can be reinvested in more interventions. The municipality saves SEK 700,000.

Pilot design – three regional funds first

Pilot phase 1 (years 1-5): Three social investment funds are started in different parts of the country—one in a metropolitan region (e.g., Västra Götaland), one in a medium-sized region (e.g., Jönköping), and one in a sparsely populated region (e.g., Norrbotten).

Organizational model:

- The fund is an **independent foundation** with a board consisting of representatives from the region, municipalities, civil society, and potential private donors.
- Start-up capital: SEK 50 million per fund (total SEK 150 million) from the state (Ministry of Health and Social Affairs) and participating regions/municipalities (2:1 matching—the state gives SEK 2 for every SEK 1 from the region/municipality).
- The fund hires a **Chief Investment Officer** (with experience from social work or impact investing) and a **Head of Evaluation** (statistician/economist).
- The fund can also accept private donations or social impact bonds (if a private investor wants to join and share the risk).

The investment process:

1. The fund issues a call for proposals for preventive projects. Applications are evaluated based on potential social return, scientific design, and the provider's capacity.
2. Approved projects receive a venture capital investment (loan or conditional grant). The project must have a clear measurement plan and a control group (randomized or matched).
3. Independent evaluators follow the project for 1–3 years and calculate the social return.
4. Upon positive return, a predetermined share is repaid to the fund. The fund reinvests in new projects.
5. Failed projects are written off—the fund bears the risk, but the lessons learned are shared.

The funds' growth goals:

- Year 3: SEK 150 million in loaned capital, 10–20 projects
- Year 5: SEK 300 million in loaned capital, self-sustaining through repayments (the fund's management costs are covered by the returns)
- Year 10: The fund has financed 100+ projects and generated a societal savings of SEK 500+ million

Evaluation of the pilot:

- Measure the funds' "social return" (SROI – Social Return on Investment). Goal: every invested krona generates at least 3 kronor in societal savings.
- Also measure systemic effects: Has the fund led public actors to start working more preventively? Have silos between administrations been broken down?
- Compare with regions that do not have social investment funds (control group).

Budget and financing

Item	Amount (one-time)	Annual operation
Start-up capital (3 funds × SEK 50 million)	SEK 150 million	–

Item	Amount (one-time)	Annual operation
Of which the state (SEK 100 million)	SEK 100 million	–
Of which regions/municipalities (SEK 50 million)	SEK 50 million	–
The funds' management costs (3 × SEK 2 million/year)	–	SEK 6 million
Independent evaluation (national)	–	SEK 4 million
Total first year	**SEK 150 million**	**SEK 10 million**

Financing: The start-up capital is taken from the Ministry of Health and Social Affairs' appropriation for "welfare innovation" (currently about SEK 500 million/year). The operating costs (SEK 10 million/year) are a fraction of the expected savings. If the funds generate an SROI of 3:1 (conservative), society saves SEK 450 million per year after a few years—an enormous societal return.

Regulatory changes required

1. **New law on social investment funds** – defines the legal form (independent foundation with the purpose of financing preventive social interventions). Gives the funds the right to receive repayments from public actors (today there is no such mechanism—one municipality cannot "get back" money from another administration).
2. **Amendment to the Local Government Act (2017:725)** – allows municipalities to enter into agreements on "social repayments" where savings are shared with an external fund.
3. **Public Procurement Ordinance (LOU)** – exemption for social investment funds: projects financed by the fund do not need to be procured according to the LOU because they are "research and development" (exemption is already possible, but is clarified).

Expected resistance and answers

"This is privatizing welfare – private investors should not make money from people's misery."

Answer: The fund is a non-profit **foundation**. Any returns are reinvested in new preventive interventions—not distributed as dividends. Private donors can donate money, but do not receive interest or profit. The model is sometimes called a "social impact bond" or "pay for success"—and has been used successfully in several countries without eroding welfare.

"It's impossible to measure social return – not everything can be reduced to numbers."

Answer: Most things can be measured if you are meticulous. Randomized control groups are the gold standard. But even simpler methods (matched comparisons, difference-in-differences) work. The important thing is to be transparent: report both quantitative and qualitative results. We already have investments based entirely on gut feeling—and they work poorly.

"This will become a way for municipalities to push costs onto the state."

Answer: On the contrary—the municipalities match the state's financing (1:2) and take on a portion of the risk. And because the savings are shared (50% to the fund, 50% to the municipality/region), the municipalities have an incentive to participate actively. It is a win-win.

"Are there any successful examples?"

Answer: Yes, several. The UK's "Social Impact Bond" (SIB) in Peterborough reduced recidivism in crime by 9% and gave investors a 3% return (and society large savings). The US's "Pay for Success" models have been used for everything from foster care placements to asthma treatment. Sweden so far only has a few small pilots (e.g., the "Uppsala model" for young adults). It is time to scale up.

Swedish examples to build upon

- **The Uppsala model (social investment funds for youth)** – a small pilot where private investors stepped in and received money back upon reduced benefit dependency. Worked technically but was too small.
- **Health and Social Care College (Vård- och omsorgscollege)** – collaboration between municipalities and private actors to train staff. A pioneer in the partnership model.
- **Almi's social loans** – Almi has lent money to social enterprises, but without the clear repayment connection to public savings.
- **Denmark's "Sociale Investeringsfonde"** – Denmark has had a national social investment fund since 2018 with good results. Sweden can learn and do better.

What you can do tomorrow

If you are a municipal or regional commissioner: Write a letter of intent with two other municipalities/regions to jointly start a social investment fund. Ask for state co-financing (2:1). Use the text above as a basis.

If you are an official in social services, the labor market, or public health: Identify a preventive intervention that you already know works (but cannot get funding for). Calculate the savings. Send an idea to a potential future fund.

If you are a philanthropist or entrepreneur: Contact a region or municipality and offer to help start a social investment fund as a foundation—you can contribute start-up capital (donation) and/or board expertise.

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